

1 ENGROSSED SENATE AMENDMENT
TO

2 ENGROSSED HOUSE
3 BILL NO. 3644

By: Stinson and Archer of the
House

4 and
5 Hines of the Senate

6
7 [venous thromboembolism - hospitals - training -
8 State Department of Health - registry -
9 requirements - reports - definitions - rules -
10 nursing care - adult daycare - information -
11 nursing aides - noncodification - effective date -
12 emergency]

13
14
15 AUTHOR: Add the following Senate Coauthor: Mann

16 AMENDMENT NO. 1. Page 1, strike the stricken title, enacting clause
17 and entire bill and insert

18 "An Act relating to venous thromboembolism; creating
19 the Blake Burgess Act; providing short title;
20 requiring specified facilities to implement certain
21 policies and training regarding venous
22 thromboembolisms; providing certain construction;
23 requiring the State Department of Health to contract
24 for establishment of a statewide venous
thromboembolism registry; requiring certain hospitals
to report specified information to registry; listing
data to be reported; requiring certain entity to
collect and report specified data; requiring
submission of certain report by the Department;
providing requirements for report; limiting use of

1 registry data; amending 63 O.S. 2021, Section 1-
2 890.2, which relates to definitions used in the
3 Continuum of Care and Assisted Living Act; adding
4 definitions; amending 63 O.S. 2021, Section 1-890.3,
5 as amended by Section 1, Chapter 357, O.S.L. 2025 (63
6 O.S. Supp. 2025, Section 1-890.3), which relates to
7 promulgation of rules; requiring certain assessment
to include specified screening; requiring assisted
living centers to make available certain consumer
information pamphlet; providing for noncodification;
providing for codification; providing an effective
date; and declaring an emergency.

8 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

9 SECTION 1. NEW LAW A new section of law not to be
10 codified in the Oklahoma Statutes reads as follows:

11 This act shall be known and may be cited as the "Blake Burgess
12 Act".

13 SECTION 2. NEW LAW A new section of law to be codified
14 in the Oklahoma Statutes as Section 1-630 of Title 63, unless there
15 is created a duplication in numbering, reads as follows:

16 A. Each hospital with an emergency department and each
17 ambulatory surgical center shall:

18 1. Develop and implement policies and procedures for the
19 rendering of appropriate medical attention for persons at risk of
20 forming venous thromboembolisms which reflect evidence-based best
21 practices relating to, at a minimum:

22 a. assessing patients for risk of venous thromboembolism
23 using a nationally recognized risk assessment tool,
24 and

1 b. treatment options for a patient diagnosed with venous
2 thromboembolism; and

3 2. Train all nonphysician personnel at least annually on the
4 policies and procedures developed under this section. For purposes
5 of this section, the term "nonphysician personnel" means all
6 personnel of the licensed facility working in clinical areas and
7 providing patient care, except those persons licensed as health care
8 practitioners.

9 B. Nothing in this section shall be construed to allow or
10 require personnel to practice outside the scope of their
11 certification, license, or qualification.

12 SECTION 3. NEW LAW A new section of law to be codified
13 in the Oklahoma Statutes as Section 1-631 of Title 63, unless there
14 is created a duplication in numbering, reads as follows:

15 A. The State Department of Health shall contract with the
16 state-designated entity for health information exchange to establish
17 and maintain a statewide venous thromboembolism registry to ensure
18 that the performance measures required to be submitted under
19 subsection B of this section are maintained and available for use to
20 improve or modify the venous thromboembolism care system, ensure
21 compliance with nationally recognized guidelines, and monitor venous
22 thromboembolism patient outcomes.

23 B. Beginning July 1, 2027, each hospital with an emergency
24 department shall electronically report annually to the statewide

1 venous thromboembolism registry information containing nationally
2 recognized venous thromboembolism measures and data on the incidence
3 and prevalence of venous thromboembolisms. Such data shall include
4 the following information:

5 1. The number of venous thromboembolisms identified and
6 diagnosed;

7 2. The age of the patient;

8 3. The ZIP code of the patient;

9 4. The sex of the patient;

10 5. Whether the patient is a resident of a licensed nursing
11 facility or assisted living center;

12 6. Whether the venous thromboembolism was fatal;

13 7. How the diagnosis was made, such as by using imaging
14 modalities; and

15 8. The treatment that was recommended for the venous
16 thromboembolism.

17 C. The Department shall require the state-designated entity for
18 health information exchange to collect data from each hospital with
19 an emergency department on the performance measures required under
20 subsection B of this section. The state-designated entity shall
21 provide to the Department regular reports on the data collected.

22 D. By June 1, 2027, the Department shall electronically submit
23 to the Governor, the President Pro Tempore of the Senate, and the
24 Speaker of the House of Representatives a detailed report on the

1 incidence of venous thromboembolism using inpatient and outpatient
2 data for services provided between the effective date of this act
3 and June 30, 2027. The report shall provide analyses of all of the
4 following:

5 1. Age category, initial primary diagnosis and procedure, and
6 secondary diagnoses, readmission rates for inpatients, admission
7 rates for venous thromboembolism for which the patient had an
8 ambulatory surgery procedure, and emergency department visits for
9 venous thromboembolism linked to any previous admission;

10 2. Whether the venous thromboembolism was present upon
11 admission;

12 3. The incidence of venous thromboembolism procedures reported
13 on the website of the Department; and

14 4. The principal payor, the sex of the patient, and the
15 patient's discharge status.

16 E. The state-designated entity for health information exchange
17 operating the registry shall only use or publish information from
18 the registry for the purposes of advancing medical research or
19 medical education in the interest of reducing morbidity or
20 mortality.

21 SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is
22 amended to read as follows:

23 Section 1-890.2. As used in the Continuum of Care and Assisted
24 Living Act:

1 1. "Assisted living center" means any home or establishment
2 offering, coordinating or providing services to two or more persons
3 who:

- 4 a. are domiciled therein,
- 5 b. are unrelated to the operator,
- 6 c. by choice or functional impairments, need assistance
7 with personal care or nursing supervision,
- 8 d. may need intermittent or unscheduled nursing care,
- 9 e. may need medication assistance, and
- 10 f. may need assistance with transfer and/or ambulation;

11 2. "Board" means the State Board of Health;

12 3. "Commissioner" means the State Commissioner of Health;

13 4. "Continuum of care facility" means a home, establishment or
14 institution providing nursing facility services as defined in
15 Section 1-1902 of this title and one or both of the following:

- 16 a. assisted living center services as defined in the
17 Continuum of Care and Assisted Living Act, and
- 18 b. adult day care center services as defined in Section
19 1-872 of this title; ~~and~~

20 5. "Department" means the State Department of Health;

21 6. "Pulmonary embolism" (PE) means a condition in which part of
22 a clot breaks off and travels to the lungs, possibly causing death;
23 and

1 7. "Venous thromboembolism" (VTE) means deep vein thrombosis
2 (DVT), which is a blood clot located in a deep vein, usually in the
3 leg or arm. The term can be used to refer to DVT, pulmonary
4 embolism, or both.

5 SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as
6 amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025,
7 Section 1-890.3), is amended to read as follows:

8 Section 1-890.3. A. The State Commissioner of Health shall
9 promulgate rules necessary to implement the provisions of the
10 Continuum of Care and Assisted Living Act. Such rules shall
11 include, but shall not be limited to:

12 1. A uniform comprehensive resident screening instrument to
13 measure the needs and capabilities of residents in all settings and
14 to determine appropriate placements of residents. Such assessment
15 shall include screening for the signs and symptoms of venous
16 thromboembolism (VTE) and techniques for providing an emergency
17 response;

18 2. Physical plant requirements meeting construction and life
19 safety codes, with provisions accommodating resident privacy and
20 independence in assisted living centers and in assisted living
21 components of continuum of care facilities based on the variable
22 capabilities of residents;

1 3. Staffing levels responsive to the variable needs of
2 residents, with provisions for sharing of staff between components
3 in a continuum of care facility;

4 4. Minimum standards for resident care including, but not
5 limited to, standards pertaining to medical care and administration
6 of medications. Standards pertaining to medication administration
7 shall, at a minimum, require the assisted living center or continuum
8 of care facility to:

- 9 a. provide or arrange qualified staff to administer
- 10 medications based on the needs of residents,
- 11 b. follow medication administration orders from a
- 12 qualified health care provider,
- 13 c. ensure that medications are reviewed monthly by a
- 14 Registered Nurse or pharmacist and quarterly by a
- 15 consultant pharmacist,
- 16 d. maintain medication administration records and
- 17 document all medication administration in such
- 18 records, and
- 19 e. have medication storage and disposal policies;

20 5. Standards for measuring quality outcomes for residents;

21 6. Provisions for individualized services chosen by and
22 designed for each resident;

23 7. Provisions to prohibit facility staff from disclosing a
24 resident's financial information to third parties without written

1 consent of the resident or the designated representative of the
2 resident;

3 8. Procedures for inspections and investigations of licensed
4 entities to ensure compliance with the Continuum of Care and
5 Assisted Living Act and rules promulgated by the Commissioner;

6 9. Enumeration of resident rights and responsibilities to be
7 observed by each facility and its staff. Such resident rights shall
8 include the freedom of choice regarding any personal attending
9 physicians and all other providers of medical services and supplies,
10 providing that the minimum standards are met by the provider
11 pursuant to the Continuum of Care and Assisted Living Act, without a
12 financial penalty or fee charged by the assisted living center;

13 10. Provisions for a surety bond or deposit from each applicant
14 in an amount sufficient to guarantee that obligations to residents
15 will be performed, with provisions for reduction or waiver of the
16 surety bond or deposit when the assets of the applicant or its
17 contracts with other persons are sufficient to reasonably ensure the
18 performance of its obligations;

19 11. Provisions requiring assisted living centers to make
20 available to residents upon admission a consumer information
21 pamphlet that contains information about venous thromboembolism
22 (VTE) including, but not limited to, risk factors and recognition of
23 the signs and symptoms of VTE;
24

1 12. Provisions for the development of a consumer guide or
2 similar resource to be posted on the Internet website of the State
3 Department of Health to assist individuals and families in
4 understanding the services provided by assisted living centers and
5 to compare and select a facility;

6 ~~12.~~ 13. Provisions for posting results of routine inspections
7 and any complaint investigations of each assisted living center on
8 the Internet website of the Department. Such information shall be
9 regularly updated to include the facility's plan of correction and
10 to indicate when a violation of a licensing regulation was corrected
11 by the facility; and

12 ~~13.~~ 14. Provisions requiring execution of a plan of care and a
13 resident service contract with the resident or resident's
14 representative.

15 B. The nursing care service of a continuum of care facility
16 shall be subject to the requirements, procedures and remedies set
17 out in the Nursing Home Care Act, including provisions relating to
18 resident rights.

19 C. The adult day care component of a continuum of care facility
20 shall be subject to requirements and procedures specified under the
21 Adult Day Care Act.

22 SECTION 6. This act shall become effective July 1, 2026.

23 SECTION 7. It being immediately necessary for the preservation
24 of the public peace, health or safety, an emergency is hereby

1 declared to exist, by reason whereof this act shall take effect and
2 be in full force from and after its passage and approval."

3 Passed the Senate the 28th day of April, 2026.

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Presiding Officer of the Senate

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7 Passed the House of Representatives the ____ day of _____,
8 2026.

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Presiding Officer of the House
of Representatives

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11 nursing aides - noncodification - effective date -
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15 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

16 SECTION 8. NEW LAW A new section of law not to be
17 codified in the Oklahoma Statutes reads as follows:

18 This act shall be known and may be cited as the "Blake Burgess
19 Act".

20 SECTION 9. NEW LAW A new section of law to be codified
21 in the Oklahoma Statutes as Section 1-630 of Title 63, unless there
22 is created a duplication in numbering, reads as follows:

23 Each hospital with an emergency department and each ambulatory
24 surgical center shall:

1 1. Develop and implement policies and procedures for the
2 rendering of appropriate medical attention for persons at risk of
3 forming venous thromboembolisms which reflect evidence-based best
4 practices relating to, at a minimum:

- 5 a. assessing patients for risk of venous thromboembolism
6 using a nationally recognized risk assessment tool,
7 and
- 8 b. treatment options for a patient diagnosed with venous
9 thromboembolism; and

10 2. Train all nonphysician personnel at least annually on the
11 policies and procedures developed under this section. For purposes
12 of this section, the term "nonphysician personnel" means all
13 personnel of the licensed facility working in clinical areas and
14 providing patient care, except those persons licensed as health care
15 practitioners.

16 SECTION 10. NEW LAW A new section of law to be codified
17 in the Oklahoma Statutes as Section 1-631 of Title 63, unless there
18 is created a duplication in numbering, reads as follows:

19 A. The State Department of Health shall contract with the
20 state-designated health information exchange to establish and
21 maintain a statewide venous thromboembolism registry to ensure that
22 the performance measures required to be submitted under subsection B
23 of this section are maintained and available for use to improve or
24 modify the venous thromboembolism care system, ensure compliance

1 with nationally recognized guidelines, and monitor venous
2 thromboembolism patient outcomes.

3 B. Beginning July 1, 2027, each hospital with an emergency
4 department shall regularly report to the statewide venous
5 thromboembolism registry information containing nationally
6 recognized venous thromboembolism measures and data on the incidence
7 and prevalence of venous thromboembolisms. Such data shall include
8 the following information:

- 9 1. The number of venous thromboembolisms identified and
10 diagnosed;
- 11 2. The age of the patient;
- 12 3. The ZIP code of the patient;
- 13 4. The sex of the patient;
- 14 5. Whether the patient is a resident of a licensed nursing or
15 assisted living facility;
- 16 6. Whether the venous thromboembolism was fatal;
- 17 7. How the diagnosis was made, such as by using imaging
18 modalities; and
- 19 8. The treatment that was recommended for the venous
20 thromboembolism.

21 C. The Department shall require the state-designated health
22 information exchange to use a nationally recognized platform to
23 collect data from each hospital with an emergency department on the
24 performance measures required under subsection B of this section.

1 The state-designated health information exchange shall provide to
2 the Department regular reports on the data collected.

3 D. By July 1, 2027, the Department shall submit to the
4 Governor, the President Pro Tempore of the Oklahoma State Senate,
5 and the Speaker of the Oklahoma House of Representatives a detailed
6 report on the incidence of venous thromboembolism using inpatient
7 and outpatient data for services provided between July 1, 2026, and
8 June 30, 2027. The report shall provide analyses of all of the
9 following:

10 1. Age category, initial primary diagnosis and procedure, and
11 secondary diagnoses, readmission rates for inpatients, admission
12 rates for venous thromboembolism for which the patient had an
13 ambulatory surgery procedure, and emergency department visits for
14 venous thromboembolism linked to any previous admission;

15 2. Whether the venous thromboembolism was present upon
16 admission;

17 3. The incidence of venous thromboembolism procedures reported
18 on the website of the Department; and

19 4. The principal payor, the sex of the patient, and the
20 patient's discharge status.

21 E. The state-designated health information exchange operating
22 the registry shall only use or publish information from the registry
23 for the purposes of advancing medical research or medical education
24 in the interest of reducing morbidity or mortality.

SECTION 11. AMENDATORY 63 O.S. 2021, Section 1-890.2, is amended to read as follows:

Section 1-890.2. As used in the Continuum of Care and Assisted Living Act:

1. "Assisted living center" means any home or establishment offering, coordinating or providing services to two or more persons who:

- a. are domiciled therein,
- b. are unrelated to the operator,
- c. by choice or functional impairments, need assistance with personal care or nursing supervision,
- d. may need intermittent or unscheduled nursing care,
- e. may need medication assistance, and
- f. may need assistance with transfer and/or ambulation;

2. "Board" means the State Board of Health;

3. "Commissioner" means the State Commissioner of Health;

4. "Continuum of care facility" means a home, establishment or institution providing nursing facility services as defined in Section 1-1902 of this title and one or both of the following:

- a. assisted living center services as defined in the Continuum of Care and Assisted Living Act, and
- b. adult day care center services as defined in Section 1-872 of this title; ~~and~~

5. "Department" means the State Department of Health;

1 6. "Pulmonary embolism (PE)" means a condition in which part of
2 a clot breaks off and travels to the lungs, possibly causing death;
3 and

4 7. "Venous thromboembolism (VTE)" means deep vein thrombosis
5 (DVT), which is a blood clot located in a deep vein, usually in the
6 leg or arm. The term can be used to refer to DVT, pulmonary
7 embolism, or both.

8 SECTION 12. AMENDATORY 63 O.S. 2021, Section 1-890.3, as
9 amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025,
10 Section 1-890.3), is amended to read as follows:

11 Section 1-890.3. A. The State Commissioner of Health shall
12 promulgate rules necessary to implement the provisions of the
13 Continuum of Care and Assisted Living Act. Such rules shall
14 include, but shall not be limited to:

15 1. A uniform comprehensive resident screening instrument to
16 measure the needs and capabilities of residents in all settings and
17 to determine appropriate placements of residents;

18 2. Physical plant requirements meeting construction and life
19 safety codes, with provisions accommodating resident privacy and
20 independence in assisted living centers and in assisted living
21 components of continuum of care facilities based on the variable
22 capabilities of residents;

1 3. Staffing levels responsive to the variable needs of
2 residents, with provisions for sharing of staff between components
3 in a continuum of care facility;

4 4. Minimum standards for resident care including, but not
5 limited to, standards pertaining to medical care and administration
6 of medications. Standards pertaining to medication administration
7 shall, at a minimum, require the assisted living center or continuum
8 of care facility to:

- 9 a. provide or arrange qualified staff to administer
- 10 medications based on the needs of residents,
- 11 b. follow medication administration orders from a
- 12 qualified health care provider,
- 13 c. ensure that medications are reviewed monthly by a
- 14 Registered Nurse or pharmacist and quarterly by a
- 15 consultant pharmacist,
- 16 d. maintain medication administration records and
- 17 document all medication administration in such
- 18 records, and
- 19 e. have medication storage and disposal policies;

20 5. Standards for measuring quality outcomes for residents;

21 6. Provisions for individualized services chosen by and
22 designed for each resident;

23 7. Provisions to prohibit facility staff from disclosing a
24 resident's financial information to third parties without written

1 consent of the resident or the designated representative of the
2 resident;

3 8. Procedures for inspections and investigations of licensed
4 entities to ensure compliance with the Continuum of Care and
5 Assisted Living Act and rules promulgated by the Commissioner;

6 9. Enumeration of resident rights and responsibilities to be
7 observed by each facility and its staff. Such resident rights shall
8 include the freedom of choice regarding any personal attending
9 physicians and all other providers of medical services and supplies,
10 providing that the minimum standards are met by the provider
11 pursuant to the Continuum of Care and Assisted Living Act, without a
12 financial penalty or fee charged by the assisted living center;

13 10. Provisions for a surety bond or deposit from each applicant
14 in an amount sufficient to guarantee that obligations to residents
15 will be performed, with provisions for reduction or waiver of the
16 surety bond or deposit when the assets of the applicant or its
17 contracts with other persons are sufficient to reasonably ensure the
18 performance of its obligations;

19 11. Assisted living facilities shall provide a consumer
20 information pamphlet to residents upon admission. The pamphlet
21 shall contain information about venous thromboembolism (VTE), risk
22 factors, and how residents can recognize the signs and symptoms of
23 VTE;
24

1 12. Provisions for the development of a consumer guide or
2 similar resource to be posted on the Internet website of the State
3 Department of Health to assist individuals and families in
4 understanding the services provided by assisted living centers and
5 to compare and select a facility;

6 ~~12.~~ 13. Provisions for posting results of routine inspections
7 and any complaint investigations of each assisted living center on
8 the Internet website of the Department. Such information shall be
9 regularly updated to include the facility's plan of correction and
10 to indicate when a violation of a licensing regulation was corrected
11 by the facility; and

12 ~~13.~~ 14. Provisions requiring execution of a plan of care and a
13 resident service contract with the resident or resident's
14 representative.

15 B. The nursing care service of a continuum of care facility
16 shall be subject to the requirements, procedures and remedies set
17 out in the Nursing Home Care Act, including provisions relating to
18 resident rights.

19 C. The adult day care component of a continuum of care facility
20 shall be subject to requirements and procedures specified under the
21 Adult Day Care Act.

22 SECTION 13. AMENDATORY 63 O.S. 2021, Section 1-1951, is
23 amended to read as follows:
24

1 Section 1-1951. A. The State Department of Health shall have
2 the power and duty to:

3 1. Issue certificates of training and competency for nurse
4 aides;

5 2. Approve training and competency programs including, but not
6 limited to, education-based programs and employer-based programs,
7 including those programs established pursuant to Section 223.1 of
8 Title 72 of the Oklahoma Statutes;

9 3. Determine curricula and standards for training and
10 competency programs. The Department shall require such training to
11 include a minimum of ten (10) hours of training in the care of
12 Alzheimer's patients, and for direct care staff, recognizing signs
13 and symptoms of venous thromboembolism (VTE) and techniques for
14 providing an emergency response;

15 4. Establish and maintain a registry for certified nurse aides
16 and for nurse aide trainees;

17 5. Establish categories and standards for nurse aide
18 certification and registration, including feeding assistants as
19 defined in 42 CFR Parts 483 and 488;

20 6. Exercise all incidental powers as necessary and proper to
21 implement and enforce the provisions of this section; and

22 7. Suspend or revoke any certification issued to any nurse
23 aide, if:
24

- a. the nurse aide is found to meet any of the requirements contained in subsection D of Section 1-1947 of this title,
- b. the nurse aide is found to meet any of the requirements contained in subsection C of Section 1-1950.1 of this title, or
- c. the nurse aide is found to have committed abuse, neglect or exploitation of a resident or misappropriation of resident or client property pursuant to the requirements contained in paragraph 7 of subsection ~~D~~ E of this section. The action to revoke or suspend may be included with the filing of any action pursuant to the requirements of paragraph 7 of subsection ~~D~~ E of this section.

B. The State Board of Health shall promulgate rules to implement the provisions of this section and shall have power to assess fees.

1. Each person certified as a nurse aide pursuant to the provisions of this section shall be required to pay certification and recertification fees in amounts to be determined by the State Board of Health, not to exceed Fifteen Dollars (\$15.00).

2. In addition to the certification and recertification fees, the State Board of Health may impose fees for training or education

1 programs conducted or approved by the Department, except for those
2 programs operated by the Oklahoma Department of Veterans Affairs.

3 3. All revenues collected as a result of fees authorized in
4 this section and imposed by the Board shall be deposited into the
5 Public Health Special Fund.

6 C. Only a person who has qualified as a certified nurse aide
7 and who holds a valid current nurse aide certificate for use in this
8 state shall have the right and privilege of using the title
9 Certified Nurse Aide and to use the abbreviation CNA after the name
10 of such person. Any person who violates the provisions of this
11 section shall be subject to a civil monetary penalty to be assessed
12 by the Department.

13 D. A person qualified by the Department as a certified nurse
14 aide shall be deemed to have met the requirements to work as a home
15 health aide pursuant to the provisions of the Home Care Act and
16 shall require no further licensure for performing services within
17 the scope of practice of home health aides.

18 E. 1. The State Department of Health shall establish and
19 maintain a certified nurse aide, nurse aide trainee and feeding
20 assistant registry that:

21 a. is sufficiently accessible to promptly meet the needs
22 of the public and employers, and

23 b. provides a process for notification and investigation
24 of alleged abuse, exploitation or neglect of residents

1 of a facility or home, clients of an agency or center,
2 or of misappropriation of resident or client property.

3 2. The registry shall contain information as to whether a nurse
4 aide has:

- 5 a. successfully completed a certified nurse aide training
- 6 and competency examination,
- 7 b. met all the requirements for certification, or
- 8 c. received a waiver from the Board.

9 3. The registry shall include, but not be limited to, the
10 following information on each certified nurse aide or nurse aide
11 trainee:

- 12 a. the full name of the individual,
- 13 b. information necessary to identify each individual.

14 Certified nurse aides and nurse aide trainees shall
15 maintain with the registry current residential
16 addresses and shall notify the registry, in writing,
17 of any change of name. Notification of change of name
18 shall require certified copies of any marriage license
19 or other court document which reflects the change of
20 name. Notice of change of address or telephone number
21 shall be made within ten (10) days of the effected
22 change. Notice shall not be accepted over the phone,
23 c. the date the individual became eligible for placement
24 in the registry, and

d. information on any finding of the Department of abuse, neglect or exploitation by the certified nurse aide or nurse aide trainee, including:

(1) documentation of the Department's investigation, including the nature of the allegation and the evidence that led the Department to confirm the allegation,

(2) the date of the hearing, if requested by the certified nurse aide or nurse aide trainee, and

(3) statement by the individual disputing the finding if the individual chooses to make one.

4. The Department shall include the information specified in subparagraph d of paragraph 3 of this subsection in the registry within ten (10) working days of the substantiating finding and it shall remain in the registry, unless:

a. it has been determined by an administrative law judge, a district court or an appeal court that the finding was in error, or

b. the Board is notified of the death of the certified nurse aide or nurse aide trainee.

5. Upon receipt of an allegation of abuse, exploitation or neglect of a resident or client, or an allegation of misappropriation of resident or client property by a certified nurse aide or nurse aide trainee, the Department shall place a pending

1 notation in the registry until a final determination has been made.
2 If the investigation, or administrative hearing held to determine
3 whether the certified nurse aide or nurse aide trainee is in
4 violation of the law or rules promulgated pursuant thereto, reveals
5 that the abuse, exploitation or neglect, or misappropriation of
6 resident or client property was unsubstantiated, the pending
7 notation shall be removed within twenty-four (24) hours of receipt
8 of notice by the Department.

9 6. The Department shall, after notice to the individuals
10 involved and a reasonable opportunity for a hearing, make a finding
11 as to the accuracy of the allegations.

12 7. If the Department after notice and opportunity for hearing
13 determines with clear and convincing evidence that abuse, neglect or
14 exploitation, or misappropriation of resident or client property has
15 occurred and the alleged perpetrator is the person who committed the
16 prohibited act, notice of the findings shall be sent to the nurse
17 aide and to the district attorney for the county where the abuse,
18 neglect or exploitation, or misappropriation of resident or client
19 property occurred and to the Medicaid Fraud Control Unit of the
20 Attorney General's Office. Notice of ineligibility to work as a
21 nurse aide in a long-term care facility, a residential care
22 facility, assisted living facility, day care facility, or any entity
23 that requires certification of nurse aides, and notice of any
24 further appeal rights shall also be sent to the nurse aide.

1 8. In any proceeding in which the Department is required to
2 serve notice or an order on an individual, the Department may send
3 written correspondence to the address on file with the registry. If
4 the correspondence is returned and a notation of the United States
5 Postal Service indicates "unclaimed" or "moved" or "refused" or any
6 other nondelivery markings and the records of the registry indicate
7 that no change of address as required by this subsection has been
8 received by the registry, the notice and any subsequent notices or
9 orders shall be deemed by the court as having been legally served
10 for all purposes.

11 9. The Department shall require that each facility check the
12 nurse aide registry before hiring a person to work as a nurse aide.
13 If the registry indicates that an individual has been found, as a
14 result of a hearing, to be personally responsible for abuse, neglect
15 or exploitation, that individual shall not be hired by the facility.

16 10. If the state finds that any other individual employed by
17 the facility has neglected, abused, misappropriated property or
18 exploited in a facility, the Department shall notify the appropriate
19 licensing authority and the district attorney for the county where
20 the abuse, neglect or exploitation, or misappropriation of resident
21 or client property occurred.

22 11. Upon a written request by a certified nurse aide or nurse
23 aide trainee, the Board shall provide within twenty (20) working
24 days all information on the record of the certified nurse aide or

1 nurse aide trainee when a finding of abuse, exploitation or neglect
2 is confirmed and placed in the registry.

3 12. Upon request and except for the names of residents and
4 clients, the Department shall disclose all of the information
5 relating to the confirmed determination of abuse, exploitation and
6 neglect by the certified nurse aide or nurse aide trainee to the
7 person requesting such information, and may disclose additional
8 information the Department determines necessary.

9 13. A person who has acted in good faith to comply with state
10 reporting requirements and this section of law shall be immune from
11 liability for reporting allegations of abuse, neglect or
12 exploitation.

13 F. Each nurse aide trainee shall wear a badge which clearly
14 identifies the person as a nurse aide trainee. Such badge shall be
15 furnished by the facility employing the trainee. The badge shall be
16 nontransferable and shall include the first and last name of the
17 trainee.

18 G. 1. For purposes of this section, "feeding assistant" means
19 an individual who is paid to feed residents by a facility or who is
20 used under an arrangement with another agency or organization and
21 meets the requirements cited in 42 CFR Parts 483 and 488.

22 2. Each facility that employs or contracts employment of a
23 feeding assistant shall maintain a record of all individuals, used
24 by the facility as feeding assistants, who have successfully

1 completed a training course approved by the state for paid feeding
2 assistants.

3 SECTION 14. This act shall become effective July 1, 2026.

4 SECTION 15. It being immediately necessary for the preservation
5 of the public peace, health or safety, an emergency is hereby
6 declared to exist, by reason whereof this act shall take effect and
7 be in full force from and after its passage and approval.

8 Passed the House of Representatives the 4th day of March, 2026.

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Presiding Officer of the House
of Representatives

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13 Passed the Senate the ____ day of _____, 2026.

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Presiding Officer of the Senate

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